WEAVE

CENTRE FOR INTEGRATIVE PSYCHIATRY

WEAVE PSYCHOTHERAPY — VOL. 8

Specialized Psychotherapy Modalities

IPT, EMDR, MBT, TFP, EFT, Transactional Analysis, and Supportive Therapy

IPT · EMDR 8 Phases · MBT · TFP · EFT-C · Transactional Analysis · Psychodrama

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Deep Study

Specialized Psychotherapy Modalities — Weave Psychotherapy Vol. 8

D1: Specialized Psychotherapy Modalities — Deep Study

1. Interpersonal Therapy (IPT)

Origins and Theoretical Basis

Interpersonal Therapy was developed by **Gerald Klerman and Myrna Weissman** in the 1970s–80s, originally as the psychotherapy arm of a pharmacotherapy trial for depression. It draws on the interpersonal tradition of Harry Stack Sullivan (who argued that psychiatry is the study of interpersonal relations) and the attachment work of John Bowlby. Adolf Meyer's psychobiological approach — emphasising the patient's adaptation to their social environment — is the other major intellectual root.

IPT is not a theory of personality or psychopathology. It is a **pragmatic, time-limited treatment** that focuses on the connection between the onset of symptoms and current interpersonal problems. The core assumption: whatever the biological vulnerability, depressive episodes occur in an interpersonal context, and resolving the interpersonal crisis will resolve the episode.

> EXAM PEARL
>
> IPT does not claim that interpersonal problems *cause* depression. It claims that depression occurs in an interpersonal context, and that working on the interpersonal context is the most efficient route to symptom resolution. This is a pragmatic, not an etiological, stance.

The Four Problem Areas

Problem Area	Definition	Typical Presentation	Key Techniques
Grief (complicated bereavement)	Abnormal grief reaction — delayed, distorted, or absent mourning after loss	Symptoms begin after death of significant other; patient avoids discussing the loss or is stuck in acute mourning	Facilitate mourning, reconstruct the relationship (positive and negative), help establish new relationships and interests
Role disputes	Conflicts with a significant other (spouse, parent, boss, friend) where expectations differ	Ongoing conflict, withdrawal, or impasse in a key relationship; patient and other have non-reciprocal expectations	Identify the dispute, determine the stage (renegotiation, impasse, dissolution), explore expectations, improve communication, consider alternatives
Role transitions	Difficulty adapting to a changed life circumstance	Life change — retirement, divorce, diagnosis, parenthood, immigration — with loss of old role and difficulty assuming new one	Mourn the old role, evaluate positives and negatives of both roles, develop mastery in the new role, build new social supports

Problem Area	Definition	Typical Presentation	Key Techniques
Interpersonal deficits	History of impoverished or inadequate social relationships	Social isolation, lack of close relationships, difficulty initiating or sustaining connections; often the most difficult problem area	Review past relationships for patterns, explore current patient-therapist relationship as a model, encourage social risk-taking, address social skills deficits

EXAM PEARL

The four IPT problem areas are: **G**rief, **R**ole disputes, **R**ole transitions, and interpersonal **D**eficits. Only ONE or TWO areas are selected as the treatment focus. The therapist explicitly names the problem area in the first few sessions and obtains the patient's agreement.

MNEMONIC

GRRD — Grief, Role disputes, Role transitions, Deficits. Or think: "**G**etting **R**elationships **R**ight when **D**epressed."

Three Phases of IPT

Phase 1: Initial Phase (Sessions 1–3)

- Assign the "sick role" — legitimise the patient's symptoms as a medical illness, reducing guilt and self-blame
- Conduct an **interpersonal inventory** — a systematic review of all significant current and recent relationships (quality, frequency, expectations, satisfactions, disappointments, changes)
- Identify the primary problem area(s)
- Establish the treatment contract: time limit (12–16 sessions), focus on the named problem area

Phase 2: Middle Phase (Sessions 4–12)

- Work on the specific problem area using IPT techniques
- For grief: facilitate mourning, review the relationship, encourage new activities
- For role disputes: clarify expectations, improve communication, consider options
- For role transitions: mourn the old role, develop competence in the new
- For interpersonal deficits: explore relationship patterns, use the therapeutic relationship
- Techniques: communication analysis, decision analysis, role-play, clarification of expectations, encouragement of affect

Phase 3: Termination Phase (Sessions 13–16)

- Explicitly discuss the ending of treatment
- Review progress and remaining symptoms
- Consolidate gains and discuss strategies for future interpersonal difficulties
- Reinforce the patient's competence in managing their own interpersonal world
- Address feelings about termination (loss of the therapist is itself a role transition)

> CLINICAL ANCHOR
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> The "sick role" in IPT is borrowed from Talcott Parsons. It serves a specific therapeutic function: it reduces self-blame, excuses the patient from obligations they cannot meet, and simultaneously obligates them to work toward recovery. It is assigned, not earned — the therapist actively tells the patient "you are ill."

Evidence Base

Condition	Key Evidence
Major depression	**Elkin et al. (1989) NIMH TDCRP** — the landmark trial. IPT equivalent to imipramine and superior to placebo for moderate-severe depression. IPT performed best for patients with high initial severity.
Depression maintenance	Frank et al. (1990) — monthly maintenance IPT reduced recurrence over 3 years; combined with imipramine was most effective
Bipolar disorder	Frank et al. (2005) — Interpersonal and Social Rhythm Therapy (IPSRT) as adjunct to mood stabilisers; stabilises circadian rhythms and reduces recurrence
Bulimia nervosa	Fairburn et al. (1991, 1993) — IPT equivalent to CBT at 12–month follow-up for BN, though slower to take effect
Perinatal depression	Spinelli & Endicott (2003) — effective for antepartum depression; O'Hara et al. (2000) for postpartum
Adolescent depression	Mufson et al. (2004) — IPT-A adapted for adolescents, effective in school-based settings

EXAM PEARL

The **NIMH Treatment of Depression Collaborative Research Program (Elkin et al., 1989)** is the single most important trial in psychotherapy research. It compared CBT, IPT, imipramine + clinical management, and pill placebo + clinical management across 250 patients at three sites. Key finding for IPT: it was particularly effective for patients with more severe depression, matching imipramine. Key finding for CBT: it was *less* effective than IPT and imipramine for severely depressed patients — a controversial result that spurred decades of debate.

> CLINICAL ANCHOR
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> IPT is one of the few psychotherapies recommended as a standalone first-line treatment for moderate depression in NICE guidelines. It is also the best-studied psychotherapy for depression in pregnancy, where pharmacotherapy poses fetal risks.

2. Eye Movement Desensitization and Reprocessing (EMDR)

Origins and the AIP Model

Francine Shapiro developed EMDR in 1987 after noticing that spontaneous eye movements appeared to reduce the distress of disturbing thoughts during a walk in the park. The initial RCT was published in 1989 and the therapy has since accumulated a substantial evidence base, primarily for PTSD.

The theoretical foundation is the **Adaptive Information Processing (AIP) model**. Core tenets:

- The brain has an innate information processing system that moves toward adaptive resolution (similar to how the body heals a wound)
- Traumatic experiences overwhelm this system and are stored in an unprocessed, state-specific form — with the original emotions, sensations, and beliefs intact
- These unprocessed memories are the basis of pathology: they are triggered by current stimuli and produce symptoms
- EMDR facilitates the resumption of adaptive processing, allowing the memory to be integrated into existing adaptive networks

EXAM PEARL

The AIP model distinguishes EMDR from exposure therapy. In exposure, the mechanism is habituation/extinction through prolonged engagement with feared stimuli. In EMDR, the mechanism is proposed to be accelerated information processing — the memory is transformed, not just extinguished. This distinction is theoretically important even though both approaches work for PTSD.

The Eight Phases of EMDR

Phase	Name	Purpose	Key Activities
1	**History and treatment planning**	Assessment and case conceptualisation	Identify target memories, current triggers, desired future states; assess client stability and dissociation risk
2	**Preparation**	Establish safety and teach self-regulation	Explain the procedure; teach the "safe/calm place" exercise; establish a stop signal; build therapeutic alliance
3	**Assessment**	Activate the target memory	Identify: image, negative cognition (NC), positive cognition (PC), VOC rating (1–7), emotion, SUDS (0–10), body location of disturbance
4	**Desensitization**	Process the disturbing memory	Bilateral stimulation (BLS) with sets of eye movements, taps, or tones; client reports whatever comes to mind between sets; continue until SUDS = 0 or ecological
5	**Installation**	Strengthen the positive cognition	Pair the positive cognition with the target memory during BLS; continue until VOC = 7

Phase	Name	Purpose	Key Activities
6	**Body scan**	Check for residual somatic disturbance	Client scans body while thinking of original target + PC; any tension or unusual sensation is targeted with BLS
7	**Closure**	Stabilise the client	Return to equilibrium; use safe place or relaxation if processing is incomplete; brief on what to expect between sessions (journaling encouraged)
8	**Re-evaluation**	Review at next session	Check if gains maintained, new material emerged, or additional targets needed; guides ongoing treatment planning

MNEMONIC

"Happy People Always Deserve Instant Body Care and Rest" — History, Preparation, Assessment, Desensitization, Installation, Body scan, Closure, Re-evaluation.

Key Scales

- **SUDS (Subjective Units of Disturbance Scale):** 0–10, where 0 = no disturbance and 10 = worst possible disturbance. Used during desensitization phase; processing continues until SUDS reaches 0 (or ecological valid level if complete resolution is not possible)
- **VOC (Validity of Cognition Scale):** 1–7, where 1 = "completely false" and 7 = "completely true." Used to measure how strongly the patient believes the

positive cognition. Processing during installation continues until VOC = 7

> EXAM PEARL
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> The negative cognition (NC) in EMDR is always an irrational, present-tense, self-referencing belief (e.g., "I am unsafe," "I am powerless," "I am worthless"). The positive cognition (PC) is the desired adaptive belief (e.g., "I am safe now," "I have choices," "I am worthy"). The NC is not challenged directly — it is expected to shift spontaneously through processing.

Bilateral Stimulation: Mechanism Debate

The mechanism of bilateral stimulation remains controversial. Competing hypotheses:

1. **Orienting response / relaxation** — BLS activates the parasympathetic nervous system, reducing arousal and facilitating processing
2. **Working memory taxation** — BLS competes for working memory resources, making the memory less vivid and emotional (similar to a cognitive load paradigm). Supported by van den Hout & Engelhard (2012)
3. **REM sleep analogy** — BLS mimics the eye movements of REM sleep, which is thought to consolidate and integrate emotional memories
4. **Interhemispheric interaction** — BLS facilitates communication between cerebral hemispheres. Weakly supported

> EXAM PEARL
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> The dismantling debate: is bilateral stimulation an essential ingredient or an epiphenomenon? Meta-analyses (Lee & Cuijpers, 2013) suggest that EMDR with BLS outperforms EMDR without BLS, and that BLS adds value beyond simple exposure. However, the effect is modest and the specific mechanism remains unclear. WHO and NICE both endorse EMDR for PTSD regardless of the mechanism question.

Evidence Base

- **PTSD:** Strongest evidence. Multiple meta-analyses show EMDR equivalent to trauma-focused CBT (prolonged exposure, CPT). WHO (2013), NICE (2018),

APA (2017), VA/DoD (2023) all recommend EMDR as a first-line PTSD treatment

- **Single-incident trauma:** EMDR may work faster than prolonged exposure (fewer sessions to remission) — Marcus et al. (1997), Ironson et al. (2002)
- **Complex PTSD:** Adaptations exist; evidence growing but less robust than for single-incident PTSD
- **Depression, anxiety, pain:** Emerging evidence, not yet first-line

> CLINICAL ANCHOR
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> EMDR is contraindicated or requires significant modification in patients with active suicidality, severe dissociative disorders (DID), unstable psychosis, or substance intoxication. Phase 2 preparation is particularly critical in complex trauma — some patients require weeks of stabilisation before any memory processing begins.

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3. Mentalization-Based Treatment (MBT)

Mentalization Defined

Anthony Bateman and Peter Fonagy developed MBT from the 1990s onward, grounded in Fonagy's theory of mentalization and its development in attachment relationships.

Mentalization is the capacity to understand behaviour — one's own and others' — in terms of underlying mental states (feelings, desires, beliefs, intentions). It is the ability to "hold mind in mind." Mentalization is:

- **Implicit** (automatic, fast, intuitive) and **explicit** (deliberate, reflective, verbal)
- **Self-focused** and **other-focused**
- **Cognitive** (beliefs, intentions) and **affective** (feelings, emotional resonance)
- **Internally focused** (on mental interiors) and **externally focused** (on observable features)

These four polarities create a multidimensional framework. Effective mentalization requires balance across all dimensions. BPD is characterised by imbalanced mentalization — typically dominated by implicit, externally focused, affective processing with collapse of reflective function under stress.

> EXAM PEARL
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> Fonagy's developmental theory: secure attachment is the crucible in which mentalization develops. The "marked and contingent mirroring" of the caregiver teaches the infant that its internal states are real, manageable, and communicable. When caregiving is neglectful or abusive, mentalization develops poorly — the child cannot make sense of the caregiver's hostile intentions and develops a fragile reflective capacity that collapses under arousal.

BPD as Mentalizing Failure

In BPD, mentalizing capacity is present but unstable. It collapses under emotional arousal, particularly in attachment contexts. When mentalizing fails, the patient reverts to three pre-mentalizing modes:

Pre-Mentalizing Mode	Description	Clinical Example
Psychic equivalence	Inner and outer reality are equated — what I feel IS what is real	"You hate me" (stated as absolute fact, not hypothesis)
Teleological mode	Mental states can only be understood through observable actions	"If you really cared, you would extend the session" (only physical proof counts)
Pretend mode	Mental states are decoupled from reality; talk is intellectual but disconnected from feeling	Elaborate psychological insights that have no emotional resonance or behavioural impact

> EXAM PEARL
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> The three pre-mentalizing modes — psychic equivalence, teleological mode, pretend mode — are the hallmark of MBT theory. Recognising which mode the patient has shifted into guides the therapist's intervention. In psychic equivalence, the therapist validates the feeling while gently introducing uncertainty. In teleological mode, the therapist helps translate the action demand into a mentalised request. In pretend mode, the therapist disrupts the disconnected narrative and grounds it in affect.

Treatment Structure

MBT was originally delivered as a **partial hospitalisation program** (Bateman & Fonagy, 1999):

- 18 months duration
- Individual therapy once weekly + group therapy three times weekly
- Expressive therapy (art, writing)
- Community meeting once weekly
- Followed by 18 months of outpatient MBT

Intensive outpatient MBT (MBT-IOP) and standard outpatient MBT have since been developed and tested.

Therapeutic Stance

The MBT therapist adopts a stance characterised by:

1. **Not-knowing** — genuine curiosity rather than expert interpretation. The therapist does not assume they know what the patient is feeling; they ask
2. **Curiosity** — active, empathic questioning about mental states: "What do you think was going on for you when...?"
3. **Affect focus** — consistently drawing attention to emotional experience in the moment, particularly in relation to the therapist
4. **Mentalizing the relationship** — when ruptures occur, the therapist openly explores what happened in terms of both parties' mental states

5. **Stop and rewind** — when a non-mentalizing process is detected, the therapist halts the narrative and goes back to the point where mentalization was lost

> CLINICAL ANCHOR
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> MBT therapists are trained to monitor their own mentalizing. When the therapist becomes certain, didactic, or interpretive — they have likely stopped mentalizing and started lecturing. The "not-knowing stance" is not a technique; it is a genuine acknowledgment that we cannot directly access another person's mind.

Evidence

Study	Design	Key Finding
Bateman & Fonagy (1999)	RCT, partial hospitalisation vs TAU	MBT superior on self-harm, suicide attempts, depression, hospital days at 18 months
Bateman & Fonagy (2001)	18–month follow-up of 1999 trial	MBT gains maintained and continued to improve; TAU group showed no further improvement
Bateman & Fonagy (2008)	8–year follow-up	MBT group maintained gains; 13% still met BPD criteria vs 87% in TAU
Bateman & Fonagy (2009)	RCT, outpatient MBT vs SCM	MBT superior to Structured Clinical Management for BPD
Rossouw & Fonagy (2012)	RCT, adolescent MBT-A vs TAU	MBT-A effective for self-harm in adolescents

EXAM PEARL

The Bateman & Fonagy (2008) 8–year follow-up is among the most impressive long-term outcomes in any BPD treatment trial. Only 13% of MBT-treated patients still met full BPD criteria at 8 years, compared to 87% in TAU. The effect was not just symptom reduction — it was personality change.

4. Transference-Focused Psychotherapy (TFP)

Kernberg's Object Relations Framework

Otto Kernberg developed TFP from his structural model of personality organisation. Kernberg synthesised ego psychology, object relations theory (Klein, Fairbairn, Jacobson, Mahler), and drive theory into a unified framework.

Levels of Personality Organization

Dimension	Neurotic	Borderline	Psychotic
Identity	Integrated, stable self/other representations	Identity diffusion — contradictory, split self/other images	Fragmented or merged self/other
Defences	Mature (repression, sublimation, reaction formation)	Primitive (splitting, projective identification, idealisation/devaluation, denial, omnipotence)	Psychotic (denial of reality, delusional projection, fragmentation)
Reality testing	Intact	Intact (distinguishes inner from outer) but impaired social reality testing	Impaired — cannot reliably distinguish inner from outer
Moral functioning	Integrated superego	Harsh but poorly integrated superego; contradictory moral standards	Bizarre or absent superego functioning

> EXAM PEARL
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> Kernberg's key diagnostic tool is the **structural interview** — a semi-structured assessment that evaluates identity integration, defence mechanisms, and reality testing through the patient's response to clarification, confrontation, and interpretation *during the interview itself*. If the patient becomes more organised after confrontation, they have borderline-level organisation. If they become more disorganised, they may have psychotic-level organisation.

Object Relations Dyads

The fundamental unit of Kernberg's model is the **object relations dyad**: a representation of self linked by an affect to a representation of an other. In borderline organisation, these dyads are split — all-good dyads (idealised self + idealised other + positive affect) are kept rigidly separate from all-bad dyads (worthless self + persecutory other + rage/fear).

TFP's core task is to help the patient integrate these split dyads through transference analysis. As the patient enacts these dyads in the relationship with the therapist, they can be identified, clarified, and eventually integrated.

Treatment Structure

- **Frequency:** Twice weekly, face-to-face
- **Duration:** 1–3 years
- **Treatment contract:** Explicit, detailed, addressing all anticipated acting-out (self-harm, substance use, missed sessions, between-session crises). The contract is non-negotiable — violations are addressed immediately as they reflect the activation of specific object relations dyads
- **Technique hierarchy:** (1) Clarification — "What do you mean when you say...?"; (2) Confrontation — pointing out contradictions and discrepancies; (3) Interpretation — linking the enacted dyad to its origins and its role in maintaining pathology. Transference interpretation is the primary mutative intervention

EXAM PEARL

TFP's technical strategy follows a hierarchy of priorities: (1) threats to the patient's life, (2) threats to the treatment (missing sessions, non-payment, substance use), (3) acting-out in or outside sessions, (4) contract violations, (5) narcissistic transference developments, (6) material discussed in a blocked or "as-if" manner. Life-threatening behaviour always takes precedence.

Evidence for BPD

- **Clarkin et al. (2007)** — RCT comparing TFP, DBT, and dynamic supportive therapy for BPD. All three improved, but TFP was the only treatment that significantly improved reflective function and attachment organisation
- **Doering et al. (2010)** — RCT: TFP superior to community treatment in reducing BPD symptoms, psychosocial functioning, and personality organisation
- **Giesen-Bloo et al. (2006)** — Schema-focused therapy superior to TFP in a Dutch RCT; however, the TFP group had higher dropout (46% vs 27%), which may have affected outcomes

CLINICAL ANCHOR

TFP is the most explicitly psychoanalytic of the evidence-based BPD treatments. Unlike DBT (which teaches skills) and MBT (which builds mentalizing), TFP aims for structural personality change through the analysis of internalised object relations as they are enacted in the transference. It is the treatment of choice for clinicians trained in psychodynamic methods who want an evidence-based framework for working with severe personality pathology.

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5. Emotionally Focused Therapy for Couples (EFT-C)

Theoretical Foundation

Sue Johnson (not to be confused with Leslie Greenberg's EFT for individuals) developed EFT for couples in the 1980s, grounding it in **attachment theory** (Bowlby, Ainsworth). The core premise: adult romantic relationships are

attachment bonds, and relationship distress is fundamentally **attachment distress** — the fear of losing the bond with a primary attachment figure.

Key theoretical concepts:

- **Primary emotions** vs **secondary emotions** — beneath the anger (secondary) is the fear of abandonment or the pain of feeling unworthy (primary). EFT accesses the primary emotion
- **Negative interaction cycles** — rigid, self-reinforcing patterns (pursue-withdraw, attack-attack, withdraw-withdraw) that are maintained by unmet attachment needs. The cycle, not the partner, is the enemy
- **Attachment injuries** — specific incidents where one partner failed to respond during a critical moment of need (e.g., not showing up after a miscarriage). These become loaded "traumas" in the relationship

> EXAM PEARL
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> The pursue-withdraw cycle is the most common negative interaction pattern in distressed couples. One partner (the pursuer, often anxiously attached) escalates demands and criticism to get a response. The other partner (the withdrawer, often avoidantly attached) shuts down to manage overwhelm. Each partner's strategy confirms the other's fears and intensifies the cycle.

Three Stages, Nine Steps

Stage	Focus	Steps
Stage 1: De-escalation (Steps 1–4)	Identify and de-escalate the negative cycle	1. Identify relational conflict issues 2. Identify the negative interaction cycle and attachment insecurities underlying positions 3. Access unacknowledged emotions (primary emotions) underlying interactional positions 4. Reframe the problem: the cycle is the enemy, not the partner
Stage 2: Restructuring interactions (Steps 5–7)	Create new bonding interactions based on primary attachment needs	5. Promote identification with disowned attachment needs and aspects of self; integrate these into relationship interactions 6. Promote acceptance of partner's new emotional experience 7. Facilitate expression of needs and wants; create bonding events (key change event: "softening" of the blaming partner and "re-engagement" of the withdrawing partner)
Stage 3: Consolidation (Steps 8–9)	Consolidate new positions and solutions	8. Facilitate new solutions to old relationship problems 9. Consolidate new positions and new cycle of attachment security

EXAM PEARL

The **"softening"** is the key change event in EFT. It occurs when the previously blaming/pursuing partner — having accessed their underlying vulnerability (fear, sadness, attachment need) — can express this vulnerability directly to the withdrawing partner, who is now emotionally engaged enough to respond. When the withdrawer responds with comfort, a corrective emotional experience occurs that restructures the attachment bond.

MNEMONIC

The three stages of EFT-C: **"Discover, Deepen, Do."** De-escalate (discover the cycle), restructure (deepen into primary emotions and create bonding events), consolidate (do — new solutions, new patterns).

Evidence

- **Johnson et al. (1999)** meta-analysis: large effect size (d = 1.3) for EFT-C compared to control
- **70–75% of couples** move from distress to recovery; **90% show significant improvement**
- **Gains are stable** — follow-up studies at 2 years show maintenance or continued improvement (Cloutier et al., 2002)
- Adapted for attachment injuries (Makinen & Johnson, 2006), trauma survivors (Johnson, 2002), and culturally diverse couples
- **Compared to other couples therapies:** EFT has the strongest evidence alongside Behavioral Couple Therapy (BCT), but EFT shows better maintenance of gains

CLINICAL ANCHOR

EFT-C is contraindicated when there is ongoing domestic violence with a controlling/abusive pattern — the vulnerability exposure required for change is unsafe. It can be used when there is situational couple violence (mutual, low-level) with appropriate safety planning. The therapist must assess violence type before beginning EFT.

6. Transactional Analysis (TA)

Berne's Framework

Eric Berne (1910–1970) developed Transactional Analysis in the 1950s-60s as a theory of personality, communication, and psychotherapy. Born from psychoanalysis (Berne trained under Erik Erikson and Paul Federn), TA translates psychoanalytic concepts into accessible, observable language.

Ego States

The ego state model is Berne's structural analysis — three distinct, observable patterns of thinking, feeling, and behaving:

Ego State	Description	Subdivisions	Observable Signs
Parent (P)	Behaviours, thoughts, feelings copied from parent figures	**Critical Parent (CP)** — rules, criticism, control; **Nurturing Parent (NP)** — care, permission, protection	Finger-wagging, "you should," furrowed brow (CP); comforting tone, "let me help," open arms (NP)
Adult (A)	Data-processing, reality-testing, here-and-now appraisal	No subdivisions in classical model	Thoughtful expression, factual language, "what are the options?", calm demeanour
Child (C)	Feelings and behaviours preserved from childhood	**Free Child (FC)** — spontaneous, creative, playful; **Adapted Child (AC)** — compliant, rebellious, anxious; conforming to perceived parental demands	Laughter, curiosity, creativity (FC); whining, pouting, defiance, "I can't" (AC)

EXAM PEARL

Berne's ego states are phenomenological, not hypothetical constructs. They are directly observable in the person's words, tone, posture, and facial expressions. Each ego state is a complete system — it has its own consistent pattern of thoughts, feelings, and behaviours. The goal of structural analysis is to help the patient recognise which ego state is active and whether it is appropriate to the situation.

Transactions

A transaction is a unit of social interaction: a stimulus from one person's ego state to another person's ego state, plus the response.

Type	Definition	Example	Result
Complementary	Response comes from the addressed ego state; vectors are parallel	A->A: "What time is it?" / A->A: "Three o'clock"	Communication continues smoothly
Crossed	Response comes from an unexpected ego state; vectors cross	A->A: "What time is it?" / CP->AC: "You should wear a watch!"	Communication breaks down; surprise, hurt, or conflict
Ulterior	Two messages simultaneously — a social (overt) message and a psychological (covert) message	Social: A->A "Would you like to see my report?" / Psychological: FC->FC "Would you like to come upstairs?"	The behavioural outcome is determined by the psychological, not the social, message

EXAM PEARL

Berne's **first rule of communication:** as long as transactions are complementary, communication can continue indefinitely. **Second rule:** when a crossed transaction occurs, communication is disrupted and one or both parties must shift ego states. **Third rule:** in ulterior transactions, the behavioural outcome is determined by the psychological (covert) level, not the social (overt) level.

Games

A game is a series of ulterior transactions that follow a predictable pattern and end with a **payoff** — a familiar bad feeling that confirms the player's life script. Berne described games using a formula:

Con + Gimmick = Response -> Switch -> Crossup -> Payoff

Classic games from *Games People Play* (1964):

- **"Why Don't You... Yes But"** — the person solicits advice, then rejects every suggestion. Payoff: "Nobody can help me" (confirms helplessness script)
- **"If It Weren't For You"** — blaming a partner for one's own limitations. Payoff: avoids the anxiety of testing one's capabilities
- **"Now I've Got You, You Son of a Bitch"** — setting up another person to make a mistake, then pouncing with righteous anger. Payoff: justifies rage
- **"Kick Me"** — behaving provocatively until someone retaliates, then adopting the victim position. Payoff: "Nobody likes me"

> EXAM PEARL
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> In Berne's system, a "game" is not playful — it is a repetitive, scripted interpersonal pattern with an ulterior motive and a predictable, negative emotional payoff. The payoff reinforces the person's life script. Therapy involves helping the patient recognise the game, understand the payoff, and choose direct communication instead.

Life Scripts and Stamps

- **Life script:** An unconscious life plan decided in early childhood, based on parental injunctions and early decisions. The script determines the person's overall life trajectory. Scripts can be "winning," "losing," or "going nowhere"
- **Injunctions:** Parental prohibitions from the Parent ego state of the actual parent to the Child ego state of the child. Goulding & Goulding (1976) identified 12 core injunctions: Don't be, Don't be you, Don't be a child, Don't grow up, Don't make it, Don't, Don't be important, Don't belong, Don't be close, Don't be well, Don't think, Don't feel

- **Stamps:** Accumulated bad feelings from games. When enough stamps are collected, the person cashes them in for a "free" dramatic action (explosion of rage, binge, walkout, suicide attempt). Similar to collecting grievances

> CLINICAL ANCHOR
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> TA's accessibility is its greatest strength and its limitation. The language (Parent/Adult/Child, games, scripts) is intuitive and patients readily engage with it. However, the simplicity can lead to superficial application. Effective TA work goes beyond labelling ego states — it involves deep script analysis, redecision work (Goulding & Goulding), and genuine emotional change.

Evidence

TA has less RCT evidence than the other modalities in this chapter but has:

- Meta-analysis by Novey (2002) showing positive effects
- Ohlsson (2002) — RCT for depression and anxiety, positive outcomes
- TA is widely used in organisational consulting, education, and counselling settings
- Integrated into broader relational psychotherapy approaches
- Recognised by the European Association for Psychotherapy

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7. Psychodrama

Moreno's Method

Jacob L. Moreno (1889–1974) developed psychodrama in Vienna in the 1920s-30s, making it one of the earliest forms of group psychotherapy. Moreno believed that spontaneity and creativity — the ability to respond to new situations with adequacy — are the curative factors in human development. Psychodrama externalises internal conflicts by enacting them on a "stage" with the help of group members.

Structure

Phase	Name	Purpose
Phase 1	Warm-up	Build group cohesion, select the protagonist, identify the theme. Director uses group exercises, discussions, or sculptures to increase spontaneity
Phase 2	Action	The protagonist enacts a scene (past, present, future, or fantasy) with the help of auxiliary egos (group members playing significant others). The director guides the action
Phase 3	Sharing	Group members share their personal responses (NOT feedback or analysis). The focus is on identification: "Your scene reminded me of when I..." This prevents intellectual distancing and builds connection

Core Techniques

Technique	Description	Purpose
Role reversal	Protagonist switches roles with the auxiliary ego, playing the other person	Builds empathy, reveals how the other might think/feel, generates new perspectives
Doubling	A group member stands behind the protagonist and voices unspoken thoughts/feelings	Accesses denied or unconscious material, deepens emotional engagement
Mirror technique	Protagonist watches as another group member enacts their role	Provides an outside perspective on one's own behaviour; confronts blind spots
Soliloquy	Protagonist speaks thoughts and feelings aloud to the audience while "frozen" in a scene	Externalises internal dialogue, bridges inner and outer experience
Surplus reality	Enacting scenes that never happened or cannot happen (e.g., saying goodbye to a deceased parent)	Provides corrective emotional experience, completes unfinished business
Empty chair	A chair represents an absent person; protagonist speaks to it	Originally Moreno's technique (later adopted by Perls for Gestalt therapy). Allows confrontation or dialogue with absent figures

EXAM PEARL

The empty chair technique is commonly associated with Gestalt therapy (Perls), but it was originally a psychodrama technique developed by Moreno. Moreno and Perls had significant professional rivalry. The technique serves a similar function in both modalities — facilitating contact with absent figures — but in psychodrama it occurs within a group context with auxiliary egos.

CLINICAL ANCHOR

Psychodrama is particularly powerful for patients who intellectualise or who have "talked about" their problems extensively without emotional change. The enactment bypasses verbal defences and creates an embodied, affectively charged experience. It is also effective for grief work, trauma processing, and interpersonal skill building. Contraindicated in actively psychotic or severely dissociative patients.

8. Supportive Psychotherapy

Definition and Scope

Supportive psychotherapy is the most widely practised form of psychotherapy in clinical settings, yet it receives the least attention in training. Wolberg classifies it as therapy aimed at **symptom relief and restoration of equilibrium** without attempting attitude change or personality restructuring. It strengthens existing defences rather than uncovering unconscious material.

EXAM PEARL

Supportive psychotherapy is NOT the absence of technique. It is a deliberate, structured approach with specific techniques, indications, and goals. The misconception that it is "just chatting" or "what you do when you can't do real therapy" is both incorrect and harmful. Supportive therapy is the treatment of choice for many patients.

Techniques

Technique	Description	Mechanism
Ventilation	Encouraging the patient to express feelings freely (catharsis)	Emotional release, reduced internal pressure, destigmatisation
Reassurance	Providing realistic comfort about prognosis, normalcy of feelings, or capacity to cope	Reduces anxiety, counters catastrophising, instils hope
Advice/guidance	Direct suggestions about behaviour or decisions	Fills an executive function gap; appropriate when the patient's judgment is temporarily impaired
Praise/encouragement	Acknowledging the patient's strengths, progress, and efforts	Reinforces adaptive behaviour, counters demoralisation
Anticipatory guidance	Preparing the patient for upcoming stressors or challenges	Reduces anxiety of the unknown, builds sense of mastery, prevents crisis
Clarification	Helping the patient understand their situation more clearly (without unconscious interpretation)	Cognitive reorganisation, reduced confusion
Suggestion	Using the therapist's authority to influence behaviour or attitudes	Leverages the therapeutic relationship for symptom relief
Environmental intervention	Helping modify the patient's environment (referrals, letters, family meetings)	Reduces external stressors, increases support

Indications

- Acute crisis (when ego function is overwhelmed)

- Chronic severe mental illness (schizophrenia, severe bipolar) as adjunct to pharmacotherapy
- Low ego strength / limited psychological mindedness
- Patients undergoing medical procedures or terminal illness
- Personality disorders where insight-oriented work is destabilising
- Between episodes: maintaining function and preventing relapse

Supportive vs Expressive: A Continuum

Dimension	Supportive Pole	Expressive Pole
Defences	Strengthens existing defences	Analyses and modifies defences
Therapist activity	More active, directive	Less active, interpretive
Focus	External stressors, coping	Internal conflicts, unconscious
Transference	Positive transference maintained, not interpreted	Transference analysed and interpreted
Regression	Discouraged	Tolerated or encouraged
Goals	Symptom relief, improved functioning	Personality change, insight
Patient selection	Lower ego strength, acute crisis, psychosis	Higher ego strength, psychological mindedness

EXAM PEARL

In practice, most psychotherapy lies on a **supportive-expressive continuum** rather than being purely one or the other. Even in psychoanalysis, supportive elements are present (the holding environment). Even in supportive therapy, moments of insight occur. The skill lies in knowing when to shift along the continuum based on the patient's moment-to-moment needs.

> CLINICAL ANCHOR
>
> The key error in supportive psychotherapy is being so supportive that the patient becomes dependent and never develops their own coping. The goal is always to strengthen the patient's autonomous functioning. Praise adaptive coping, reinforce problem-solving attempts, and gradually reduce the frequency of sessions as the patient stabilises.

Evidence

- Historically used as a control condition in psychotherapy trials, which paradoxically demonstrated it was more effective than expected
- **De Jonghe et al. (2001)** — supportive psychotherapy for depression equivalent to psychodynamic psychotherapy at 6 months
- **Hellerstein et al. (1998)** — supportive psychotherapy effective for Cluster C personality disorders
- **Clarkin et al. (2007)** — dynamic supportive therapy showed improvement in BPD (though less than TFP on some measures)
- **NICE guidelines** recommend supportive counselling as an option for mild depression

Comparative Tables

Table 1: All Specialised Modalities at a Glance

Modality	Founder	Core Mechanism	Format	Duration	Primary Evidence
IPT	Klerman & Weissman	Interpersonal context of depression	Individual; 12–16 sessions	3–4 months	Depression, bulimia, perinatal depression
EMDR	Shapiro	Adaptive Information Processing	Individual; 6–12 sessions typically	1–3 months	PTSD (first-line)
MBT	Bateman & Fonagy	Restoring mentalizing capacity	Individual + group; 18 months	18–36 months	BPD (partial hospitalisation and outpatient)
TFP	Kernberg	Integrating split object relations via transference	Individual; twice weekly	1–3 years	BPD, personality organisation
EFT-C	Sue Johnson	Restructuring attachment bonds via primary emotion	Couples; 8–20 sessions	3–6 months	Couple distress (d = 1.3)
TA	Berne	Script analysis, ego state awareness	Individual or group	Variable	Depression, anxiety, organisational
Psychodrama	Moreno	Spontaneity, role enactment, catharsis	Group	Variable	Trauma, grief, interpersonal skills

Modality	Founder	Core Mechanism	Format	Duration	Primary Evidence
Supportive	Multiple	Defence strengthening, coping enhancement	Individual	Variable	Chronic mental illness, crisis, low ego strength

Table 2: BPD Treatment Comparison (DBT vs MBT vs TFP vs Schema

Therapy)

Dimension	DBT	MBT	TFP	Schema Therapy
Founder	Linehan	Bateman & Fonagy	Kernberg	Young
Theoretical basis	Biosocial theory; dialectics; Zen	Attachment + mentalization	Object relations; structural model	Cognitive-developmental ; attachment
Core target	Emotion dysregulation + behavioural patterns	Mentalizing failure	Split object relations / identity diffusion	Early maladaptive schemas + maladaptive coping modes
Stance	Dialectical (acceptance + change); coaching	Not-knowing; curiosity; mentalizing stance	Technically neutral; clarification-confrontation-interpretation	Limited reparenting; empathic confrontation
Key technique	Skills training (4 modules), diary card, chain analysis	Mentalizing the moment; stop-and-rewind	Transference interpretation	Imagery rescripting, mode dialogues, chair work
Format	Individual + skills group + phone coaching	Individual + group (in full model)	Individual (twice weekly)	Individual (1–2× weekly)
Duration	12+ months standard	18 months (original); 12–18 outpatient	1–3 years	1–3 years

Dimension	DBT	MBT	TFP	Schema Therapy
Hierarchy	Life-threatening > therapy-interfering > quality of life > skills	No rigid hierarchy; mentalizing always the focus	Life threats > treatment threats > acting out > narcissistic transference	Schema activation guides session focus
Key RCT	Linehan et al. (1991, 2006)	Bateman & Fonagy (1999, 2009)	Clarkin et al. (2007); Doering et al. (2010)	Giesen-Bloo et al. (2006); Farrell et al. (2009)
Unique strength	Most evidence; skills generalisable; structured for suicidal patients	Long-term personality change; group format	Only treatment shown to change personality organisation and attachment	Most accepted by patients (lowest dropout in RCTs)
Dropout rate	~25%	~15–25%	~35–46%	~20–27%

EXAM PEARL

When asked to compare BPD treatments in a viva, organise your answer by: (1) theoretical basis, (2) therapeutic stance, (3) primary mechanism of change, (4) key technique, (5) one landmark trial. All four are evidence-based and effective. They differ in theory and technique but share common therapeutic factors: a structured treatment framework, clear focus on the therapeutic relationship, and explicit management of self-harm.

EXAM STRATEGY

For exam answers on specialised modalities, structure your response as: (1) founder and year, (2) theoretical basis in one sentence, (3) core mechanism, (4) key techniques (name 3), (5) landmark trial with result, (6) primary indication. This gives a complete answer in under 2 minutes.

EXAM STRATEGY

When comparing modalities, use a table format in your written answer. Examiners consistently give higher marks for structured, comparative answers over narrative descriptions. Name the founder, the mechanism, and one trial for each modality.

EXAM STRATEGY

The most commonly examined topics in this chapter are: (1) IPT problem areas, (2) EMDR phases, (3) BPD treatment comparison, (4) the MBT concept of mentalization, (5) supportive vs expressive psychotherapy continuum. If you can speak fluently on these five topics, you will handle most questions.

EXAM STRATEGY

For EMDR, examiners often ask about mechanism of action and the controversy around bilateral stimulation. A balanced answer acknowledges the strong evidence for EMDR's efficacy while noting that the specific mechanism (and whether BLS is essential vs incidental) remains debated. Avoid taking a strong position either way.

CLINICAL ANCHOR

In clinical practice, most patients receive a blend of modalities tailored to their needs, not a "pure" form of any single therapy. Wolberg's central thesis holds: the therapeutic relationship and non-specific factors contribute more to outcome than the specific technique. Knowledge of multiple modalities allows the clinician to choose the right tool at the right moment.

CLINICAL ANCHOR

The most common error with specialised modalities is applying them to inappropriate patients. IPT requires a definable interpersonal problem area. EMDR requires a specific traumatic memory to target. MBT and TFP require patients who can tolerate a therapeutic relationship without immediate flight. EFT-C requires both partners to be present and non-violent. Supportive therapy is the fallback when more intensive modalities are contraindicated.

CLINICAL ANCHOR

Training matters enormously for specialised modalities. EMDR, MBT, TFP, and EFT-C all require certified training and supervision beyond reading a manual. TA has its own international certification body. The modalities described here cannot be safely self-taught from a textbook.

CLINICAL ANCHOR

Supportive psychotherapy is the most commonly delivered and most underrated modality. Every psychiatrist — regardless of psychotherapy orientation — should be competent in supportive techniques because every patient encounter involves supportive elements. It is particularly important for residents who will spend most of their careers providing pharmacotherapy within a supportive psychotherapeutic framework.

02

Clinical Quick Reference

Specialized Psychotherapy Modalities — Weave Psychotherapy Vol. 8

D6: Specialized Psychotherapy Modalities — Clinical Quick Reference

1. IPT — Four Problem Areas

Problem Area	Definition	Stage/Status Assessment	Key Techniques	When to Choose
Grief	Abnormal bereavement — delayed, distorted, or absent	Is mourning blocked, distorted, or chronic?	Facilitate mourning; reconstruct the relationship (positive + negative); encourage new activities/relationships	Symptoms began after a death; patient avoids or is stuck in grief
Role disputes	Conflicting expectations with a significant other	Renegotiation (both still trying), impasse (stuck), dissolution (irreparable)	Clarify expectations; communication analysis; role-play; decision analysis	Ongoing interpersonal conflict with identifiable other
Role transitions	Difficulty adapting to life change	Old role vs new role — what is lost, what is gained?	Mourn old role; evaluate both roles realistically; develop mastery in new role; build new supports	Major life change (retirement, diagnosis, divorce, migration, parenthood)
Interpersonal deficits	Impoverished relationships throughout life	History of isolation; few or no close relationships	Review past relationship patterns; use therapeutic relationship as model; encourage social risk-taking	No clear grief, dispute, or transition; lifelong pattern of isolation

EXAM PEARL

Three phases of IPT: Phase 1 (sessions 1–3) — sick role, interpersonal inventory, identify problem area, contract. Phase 2 (sessions 4–12) — work on problem area. Phase 3 (sessions 13–16) — termination, consolidate gains, address ending as a role transition. Landmark trial: Elkin et al. (1989) NIMH TDCRP — IPT equivalent to imipramine for moderate-severe depression.

2. EMDR — Eight Phases

Phase	Name	Key Activity	Target
1	History & treatment planning	Identify targets (past memories, current triggers, future templates)	Case conceptualisation
2	Preparation	Safe place exercise, explain procedure, stop signal	Client stability
3	Assessment	Image + NC + PC + VOC (1–7) + emotion + SUDS (0–10) + body location	Activate memory network
4	Desensitization	BLS sets; "What comes up?"; continue until SUDS = 0	Process disturbance
5	Installation	Pair PC with target during BLS; continue until VOC = 7	Strengthen adaptive belief
6	Body scan	Scan for residual tension with target + PC in mind; BLS any disturbance	Clear somatic residue
7	Closure	Stabilise; safe place if incomplete; brief on between-session expectations	Return to equilibrium
8	Re-evaluation	Next session: check maintenance, new material, additional targets	Guide treatment planning

Key scales: SUDS (0–10, disturbance), VOC (1–7, belief validity). NC is always present-tense, self-referencing, irrational ("I am powerless"). PC is the desired adaptive belief ("I have choices").

> EXAM PEARL
>
> AIP model: trauma overwhelms innate processing -> stored in state-specific form -> triggered by current stimuli -> symptoms. EMDR resumes adaptive processing. Mechanism of BLS debated: working memory taxation (van den Hout), orienting response, REM analogy. Evidence: equivalent to TF-CBT for PTSD (WHO, NICE, APA first-line).

3. MBT — Key Concepts

Concept	Definition
Mentalization	Understanding behaviour in terms of underlying mental states (feelings, beliefs, desires, intentions) — "holding mind in mind"
Four dimensions	Implicit/explicit, self/other, cognitive/affective, internal/external
Psychic equivalence	Inner = outer; "I feel it, so it must be true" (pre-mentalizing mode)
Teleological mode	Only actions count as proof of mental states: "If you cared, you'd extend the session"
Pretend mode	Intellectualised talk disconnected from affect; sounds insightful but changes nothing
Not-knowing stance	Genuine curiosity; the therapist does not assume they know the patient's mind
Stop and rewind	When mentalizing breaks down, halt and return to the point it was lost
Affect focus	Consistently direct attention to emotional experience in the here-and-now

Format: Individual (1×/week) + group (3×/week) in original model; 18 months. Outpatient adaptations exist.

Evidence: Bateman & Fonagy (1999) RCT — superior to TAU for self-harm, suicide attempts, hospital days. 8–year follow-up (2008): only 13% still met BPD criteria vs 87% TAU.

4. TFP — Structural Interview and Object Relations

Kernberg's Three Levels

Dimension	Neurotic	Borderline	Psychotic
Identity	Integrated	Diffuse (split self/other images)	Fragmented/merged
Defences	Mature (repression, sublimation)	Primitive (splitting, projective identification, idealisation/devaluation)	Psychotic (delusional projection)
Reality testing	Intact	Intact but impaired social reality	Impaired

TFP Technique Hierarchy

1. Threats to life
2. Threats to treatment (absences, substance use)
3. Acting out in/outside sessions
4. Contract violations
5. Narcissistic transference
6. Material in "as-if" manner

Key technique: Clarification -> Confrontation -> Transference interpretation (the primary mutative intervention).

Format: Twice weekly, face-to-face, 1–3 years. Detailed treatment contract addressing all anticipated acting-out.

Evidence: Clarkin et al. (2007) — only BPD treatment shown to change reflective function and attachment organisation. Doering et al. (2010) — superior to community treatment.

5. EFT-C — Three Stages, Nine Steps

Stage	Steps	Focus	Key Change Event
1. De-escalation	1. Identify conflict issues 2. Identify negative cycle + attachment insecurities 3. Access primary (unacknowledged) emotions 4. Reframe: the cycle is the enemy	Surface the cycle	Couple sees the pattern, not each other, as the problem
2. Restructuring	5. Promote identification with disowned needs 6. Promote acceptance of partner's new experience 7. Facilitate bonding events	Create new attachment interactions	**Softening** — blaming partner expresses vulnerability; withdrawer responds with comfort
3. Consolidation	8. New solutions to old problems 9. Consolidate new positions and secure cycle	Cement new patterns	New, secure interaction cycle replaces old negative cycle

Theoretical basis: Attachment theory (Bowlby). Adult romantic bonds = attachment bonds. Distress = attachment insecurity. Pursue-withdraw is the most common negative cycle.

Evidence: d = 1.3 (Johnson et al., 1999 meta-analysis). 70–75% recovery rate. Gains stable at 2-year follow-up. Contraindicated in controlling domestic violence.

6. Transactional Analysis — Ego States and Transactions

Ego States

State	Description	Subdivisions
Parent (P)	Copied from parental figures	Critical Parent (rules, control) / Nurturing Parent (care, permission)
Adult (A)	Here-and-now reality testing	No subdivisions
Child (C)	Preserved childhood patterns	Free Child (spontaneous) / Adapted Child (compliant or rebellious)

Transaction Types

Type	Pattern	Result
Complementary	Response from addressed ego state; parallel vectors	Communication continues
Crossed	Response from unexpected ego state; vectors cross	Communication breaks down
Ulterior	Social message + covert psychological message simultaneously	Behaviour follows the psychological level

Key concepts: Games (repetitive ulterior transactions with a negative payoff), life scripts (unconscious life plan from childhood), stamps (accumulated bad feelings cashed in for dramatic action), 12 injunctions (Goulding & Goulding: Don't be, Don't be you, Don't be close, etc.).

7. BPD Treatments — Head-to-Head Comparison

Feature	DBT	MBT	TFP	Schema Therapy
Founder	Linehan	Bateman & Fonagy	Kernberg	Young
Theory	Biosocial + dialectics	Attachment + mentalization	Object relations	Cognitive-developmental
Core target	Emotion dysregulation	Mentalizing failure	Split object relations	Early maladaptive schemas
Stance	Dialectical (acceptance + change)	Not-knowing, curious	Technically neutral	Limited reparenting
Key technique	Skills training, chain analysis	Mentalizing the moment	Transference interpretation	Imagery rescripting, chair work
Format	Individual + group + phone	Individual + group	Individual 2×/week	Individual 1–2×/week
Duration	12+ months	18 months (original)	1–3 years	1–3 years
Landmark RCT	Linehan (1991, 2006)	Bateman & Fonagy (1999)	Clarkin et al. (2007)	Giesen-Bloo et al. (2006)
Dropout	~25%	~15–25%	~35–46%	~20–27%
Unique strength	Most evidence; skills for suicidal patients	Long-term personality change	Changes personality organisation	Lowest dropout rates

8. Evidence Summary Table

Modality	Primary Indication	Landmark Trial	Effect Size / Key Result
IPT	Major depression	Elkin et al. (1989) NIMH TDCRP	Equivalent to imipramine; best for severe depression
IPT	Bulimia nervosa	Fairburn et al. (1993)	Equivalent to CBT at 12–month follow-up
IPT	Perinatal depression	O'Hara et al. (2000)	Effective; preferred when avoiding medication
EMDR	PTSD	Multiple meta-analyses (2013–2023)	Equivalent to TF-CBT; WHO/NICE/APA first-line
MBT	BPD	Bateman & Fonagy (1999, 2008)	87% no longer met BPD criteria at 8 years
TFP	BPD	Clarkin et al. (2007)	Only treatment to improve reflective function + attachment
EFT-C	Couple distress	Johnson et al. (1999) meta-analysis	d = 1.3; 70–75% recovery rate
Supportive	Depression	De Jonghe et al. (2001)	Equivalent to psychodynamic therapy at 6 months
Supportive	BPD (as active control)	Clarkin et al. (2007)	Significant improvement (more than expected)
Psychodrama	Various (trauma, grief)	Kipper & Ritchie (2003) meta-analysis	d = 0.95 overall

9. Supportive vs Expressive Continuum

Dimension	Supportive Pole	Expressive Pole
Defences	Strengthen	Analyse and modify
Therapist activity	Active, directive	Interpretive, receptive
Focus	External stressors, coping	Internal conflicts, unconscious
Transference	Maintained, not interpreted	Analysed and interpreted
Regression	Discouraged	Tolerated
Goals	Symptom relief, functioning	Personality change, insight
Patient selection	Lower ego strength, crisis, psychosis	Higher ego strength, psychologically minded

EXAM PEARL

Supportive psychotherapy techniques: ventilation, reassurance, advice/guidance, praise, anticipatory guidance, clarification, suggestion, environmental intervention. It is the most widely practised yet most undertaught modality. Every psychiatrist must be competent in it.

10. Viva Questions

Q1: What are the four problem areas in IPT? Give a clinical example of each.

Grief (depression after husband's death, with absent mourning), role disputes (marital conflict over division of labour — stage: impasse), role transitions (depression after retirement — loss of professional identity), interpersonal deficits (chronic social isolation with no identifiable precipitant). Only 1–2 areas are selected as focus. The therapist names the area explicitly and obtains the patient's agreement.

Q2: Describe the eight phases of EMDR.

History/planning, preparation (safe place, stabilisation), assessment (image + NC + PC + VOC + SUDS + body location), desensitization (BLS until SUDS = 0), installation (strengthen PC until VOC = 7), body scan (clear somatic residue), closure (stabilise, debrief), re-evaluation (next session review). The AIP model proposes that trauma is stored in unprocessed state-specific form; EMDR resumes adaptive processing.

Q3: What is mentalization? How does it fail in BPD?

Mentalization is understanding behaviour in terms of underlying mental states — "holding mind in mind." It has four dimensions: implicit/explicit, self/other, cognitive/affective, internal/external. In BPD, mentalizing is present but unstable — it collapses under emotional arousal, especially in attachment contexts. The patient reverts to pre-mentalizing modes: psychic equivalence (inner = outer), teleological mode (only actions count), or pretend mode (intellectual but disconnected from affect).

Q4: Compare DBT, MBT, TFP, and Schema Therapy for BPD.

All four are evidence-based. DBT targets emotion dysregulation via skills training (most evidence, best for actively suicidal patients). MBT targets mentalizing failure via a curious, not-knowing stance (best long-term personality change data). TFP targets split object relations via transference interpretation (only treatment shown to change attachment organisation). Schema Therapy targets early maladaptive schemas via imagery rescripting and limited reparenting (lowest dropout rates). They share: structured framework, explicit attention to the therapeutic relationship, and protocols for managing self-harm.

Q5: What is the difference between supportive and expressive psychotherapy?

They exist on a continuum. Supportive therapy strengthens defences, uses active techniques (reassurance, advice, ventilation), maintains positive transference without interpreting it, and aims for symptom relief. Expressive

therapy analyses defences, interprets transference, tolerates regression, and aims for personality change. Patient selection: supportive for lower ego strength, acute crisis, psychosis; expressive for higher ego strength, psychological mindedness. Most clinical work involves a blend.

Q6: Describe the key change event in EFT for couples.

The "softening" — the previously blaming/pursuing partner accesses their primary emotion (fear, sadness, attachment longing beneath the anger) and expresses this vulnerability directly to the withdrawing partner, who has become emotionally re-engaged enough to respond with comfort and reassurance. This creates a corrective emotional experience that restructures the attachment bond and breaks the negative interaction cycle.

Q7: What is the structural interview in TFP?

Kernberg's diagnostic tool assessing personality organisation through three dimensions: identity integration, defence mechanisms, and reality testing. The interviewer uses clarification, confrontation, and interpretation during the interview itself and observes the patient's response. If the patient becomes more organised after confrontation, they have borderline-level organisation (identity diffusion + primitive defences + intact reality testing). If they become more disorganised, psychotic-level organisation is indicated.

Q8: Name three TA games and explain their payoffs.

"Why Don't You... Yes But" — solicits then rejects all advice; payoff: confirms "nobody can help me." "If It Weren't For You" — blames partner for own limitations; payoff: avoids testing one's own capabilities. "Kick Me" — behaves provocatively until retaliated against, then adopts victim position; payoff: confirms "nobody likes me." All games end with a familiar bad feeling that reinforces the life script.

Q9: What is the sick role in IPT and why is it therapeutically useful?

Borrowed from Talcott Parsons. The therapist explicitly tells the patient they are ill (legitimising symptoms), which reduces self-blame and guilt, excuses the patient from obligations they cannot meet, and simultaneously obligates them to work actively toward recovery. It reframes depression from a moral failing to a medical condition. Assigned in Phase 1 of IPT.

Q10: What are the core techniques of psychodrama?

Role reversal (protagonist plays the other person — builds empathy), doubling (group member voices protagonist's unspoken thoughts), mirror technique (another enacts the protagonist's role while they watch — confronts blind spots), surplus reality (enacting impossible scenes, e.g., saying goodbye to the deceased), soliloquy (speaking inner thoughts aloud), empty chair (dialogue with absent figure — originally Moreno's technique, later adopted by Perls for Gestalt therapy). Three phases: warm-up, action, sharing.

Clinical Quick Reference — Weave Psychotherapy Vol. 8

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